

AAKRITI

Converting 2D technical report into 3d clear & visualize understanding

Master Research Documentation

Doctor-Controlled AI-Assisted Medical Imaging Communication Platform

AI Assists → Doctor Verifies → Patient Understands

Version — September 2026

Executive Summary

AAkriti is a doctor-controlled, AI-assisted medical imaging communication platform that converts relevant findings from a patient's X-ray, CT or MRI into understandable visual explanations. The platform is designed to bridge the gap between clinically detailed imaging information and patient-level understanding. AI assists with localization or segmentation, the doctor verifies or edits the result, and the patient receives a modality-appropriate explanation through highlighted 2D imaging and, where suitable, patient-specific 3D/AR visualization.

AAkriti is not intended to replace doctors, provide autonomous diagnosis, recommend treatment, or predict healing time. Its central purpose is communication: helping clinicians show patients what they are explaining.

Master Positioning Statement

AAkriti is a doctor-controlled, AI-assisted medical-imaging communication platform that converts relevant findings from a patient's X-ray, CT or MRI into understandable visual explanations—using highlighted 2D views and, where technically appropriate, patient-specific 3D/AR visualization.

Core Principle

AI assists → Doctor verifies → Patient understands

1. Project Foundation

1.1 Project Name

AAkriti

1.2 Domain

Healthcare + Medical Imaging + Artificial Intelligence + 3D Visualization + Augmented Reality + Patient Communication

1.3 Primary Problem Domain

AAkriti addresses the doctor–patient communication gap in medical imaging. Medical imaging provides clinicians with detailed information, but patients may find technical reports, 2D images and medical terminology difficult to interpret. A patient may understand the words but still struggle to understand where a finding is located and how it relates to their own anatomy.

2. Problem Identification

2.1 Core Problem

The core problem is not that medical imaging is inherently inaccurate. The problem is that technically correct clinical information may remain difficult for a non-expert patient to understand when it is communicated mainly through technical reports, static 2D images and verbal explanation.

2.2 Final Problem Statement

Patients undergoing medical imaging face difficulty understanding the exact location and anatomical significance of their findings because conventional reports, 2D images and verbal explanations are designed primarily for clinical interpretation rather than intuitive patient-level visualization.

2.3 SIH-Style Framing

Patients face difficulty understanding the location and anatomical significance of medical-imaging findings because conventional reports, 2D images and verbal explanations are not designed for intuitive patient-level visualization.

3. Stakeholder Analysis

3.1 Primary Users — Doctors

- Radiologists — interpret X-ray, CT and MRI studies and may need to communicate findings.
- Orthopedic doctors — especially relevant to the initial fracture-focused MVP.
- Other referring clinicians — surgeons, oncologists, physicians and emergency doctors in future expansion.

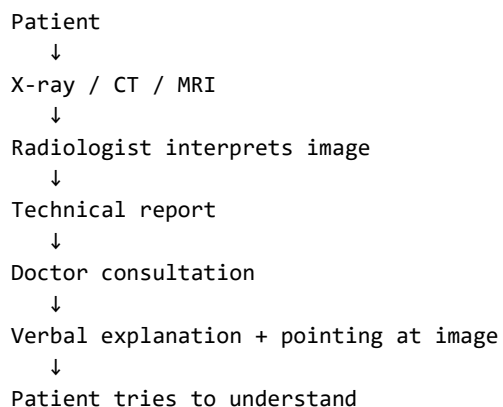
3.2 Institutional Customers

- Hospitals
- Diagnostic centres
- Radiology departments
- Orthopedic clinics
- Diagnostic chains
- Specialty healthcare centres

3.3 End Beneficiaries

- Patients
- Caregivers and family members

4. Existing Situation and Current Workflow



The weakness addressed by AAkriti is primarily at the final communication layer. The imaging can be clinically useful while the patient's understanding of the finding's location, anatomy and meaning remains incomplete.

5. Research Evidence

5.1 Patient Understanding of Imaging

A patient survey cited in the earlier research found that 45.8% of respondents correctly identified whether their imaging examination involved radiation, while approximately 19% reported unanswered questions. This supports the broader issue of incomplete patient understanding of medical imaging.

Qualification: this evidence concerns understanding of the imaging examination and should not be presented as proof that 45.8% of patients cannot understand a fracture finding.

5.2 Patient-Friendly Radiology Reporting

A patient-friendly radiology reporting study reported that 84.7% of patients said interactive plain-language definitions and diagrams helped them understand their report. Average viewing time with interactive aids was approximately 5.8 minutes compared with 1.8 minutes for raw text.

Interpretation: this supports the hypothesis that understandable visual aids can improve engagement with medical information. It does not establish AAkriti's own performance.

5.3 Radiology Communication Errors

The earlier research included a study of 380 radiology communication errors. It reported direct impact on patient care for 37.9% of errors and potential impact for 52.6%. AAkriti should use this as evidence that communication is an important workflow concern, not as proof that AAkriti will eliminate communication errors.

5.4 3D Visualization Evidence

Research included in the earlier work compared presentation formats involving no image, 2D images and 3D images. The reported findings supported the usefulness of medical images for understanding and indicated a preference for 3D visualization in the studied context.

6. Existing Solutions

Approach	Strength	Limitation / AAkriti opportunity
Verbal explanation	Already part of normal consultation	Depends heavily on patient comprehension and spatial imagination
Written radiology report	Detailed clinical documentation	Terminology and structure may be difficult for non-experts
2D scan viewing	Clinically essential	Non-experts may struggle to translate 2D information into spatial anatomy
3D printing	Physical and tangible	Time, cost and production

		requirements
Medical 3D software	Advanced visualization and analysis	Often optimized for professional/research workflows
AR/VR healthcare	Immersive visualization	Hardware/workflow complexity varies; AR itself is not unique

AAkriti should not claim that medical 3D visualization or AR was invented by the project. The opportunity is the communication workflow that connects the patient's actual imaging finding to a doctor-verified, patient-facing explanation.

7. Failure Condition

The communication workflow becomes weak when clinical information is technically correct but the patient cannot spatially understand what the doctor is referring to.

Doctor: "This is the fracture here."

Patient: understands "fracture"

but may still wonder:

- Which bone?
- Exactly where?
- How does it relate to surrounding anatomy?
- What is the doctor pointing at?

AAkriti addresses this by providing a shared visual reference between doctor and patient.

8. Research Gap

Existing research demonstrates the value of patient-friendly reports, diagrams, 3D visualization and AR-based medical communication, while AI can assist with medical-image analysis. However, a practical workflow gap remains in connecting an image-grounded, clinician-verified finding from the patient's own imaging study to a patient-facing visual explanation while adapting the visualization to the imaging modality.

8.1 Final Research Gap Statement

Existing research demonstrates the value of patient-friendly reports, diagrams and 3D/AR visualization, but a practical workflow gap remains in connecting an image-grounded, clinician-verified finding from the patient's own imaging study to a patient-facing visual explanation while adapting the visualization to the imaging modality.

9. Desired Outcome

9.1 Patient Outcome

- Understand what the doctor is showing.
- Understand where the finding is located.
- Recognize the affected anatomical structure.

- Connect technical information to a visual reference.
- Ask more informed questions during consultation.

9.2 Doctor Outcome

- Select the relevant finding.
- Show it visually.
- Explain it in simple language.
- Correct or reject AI output.
- Control what the patient sees.

9.3 Hospital Outcome

- Standardized patient-facing communication.
- Digital patient education.
- Potential differentiation of patient experience.
- Scalable communication software without specialized visualization hardware.

10. Proposed Solution

AAkriti is a doctor-controlled AI-assisted medical imaging communication platform that converts selected findings from X-ray, CT and MRI studies into patient-understandable visual explanations. AI assists with anatomical localization or segmentation, the doctor verifies and edits the result, and the patient receives a modality-appropriate explanation through highlighted 2D imaging and, where suitable, interactive 3D/AR visualization.

10.1 Product Boundaries

- AAkriti is not an AI doctor.
- AAkriti is not an autonomous diagnostic system.
- AAkriti does not replace radiologists or clinicians.
- AAkriti does not provide treatment recommendations.
- AAkriti does not use healing-time prediction as a core feature.
- AAkriti does not claim universal X-ray-to-patient-specific-3D reconstruction.

11. Modality-Aware Strategy

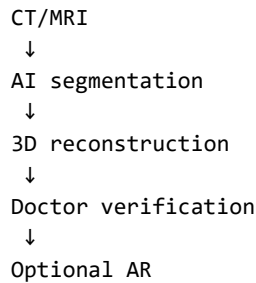
11.1 X-ray

A conventional X-ray is a 2D projection. Therefore, the initial AAkriti X-ray workflow should remain 2D-first.

```
X-ray
↓
AI localization
↓
Highlighted 2D image
↓
Doctor verification
↓
Patient explanation
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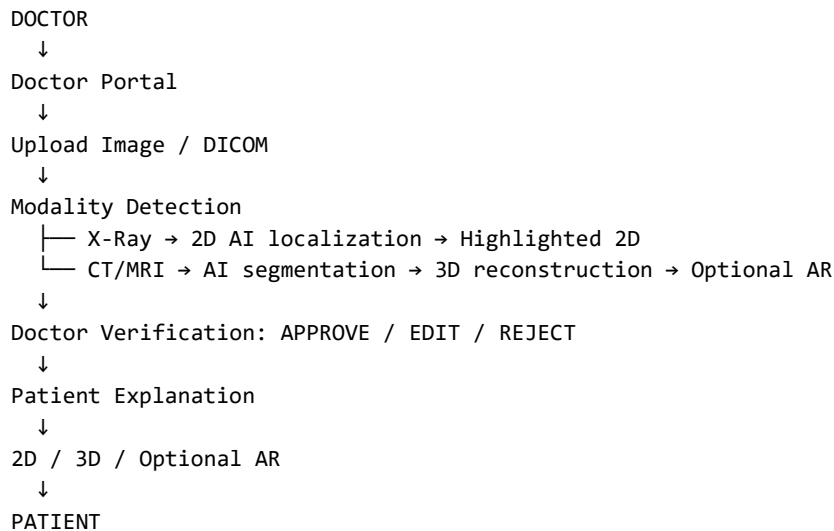
11.2 CT/MRI

CT and MRI provide volumetric or slice-based information that can support patient-specific segmentation and 3D reconstruction for suitable use cases.



Final modality strategy: X-ray → 2D explanation; CT/MRI → 3D/AR where technically and clinically appropriate.

12. Detailed Product Workflow

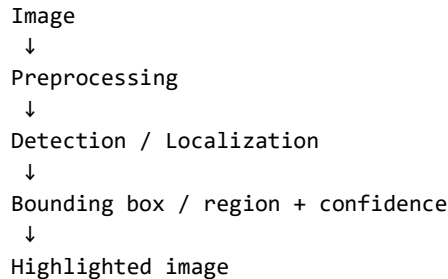


1. Doctor logs into the platform.
2. Doctor creates or selects a case.
3. Doctor uploads a medical image or DICOM study.
4. The system detects the imaging modality.
5. For X-ray, AI performs 2D localization/detection.
6. For CT/MRI, AI performs segmentation for a selected validated use case.
7. The system prepares the corresponding visualization.
8. The doctor reviews the AI result.
9. The doctor can approve, edit or reject the result.
10. Only the doctor-approved finding proceeds to the patient-facing view.
11. The patient sees the original scan, highlighted finding and simple explanation.
12. Where suitable, the patient can interact with a 3D model and optional AR visualization.

13. AI Layer

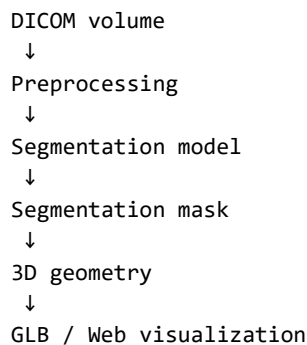
13.1 X-ray AI

Possible initial approach: YOLO-based localization/detection or another suitable pretrained fracture model.



13.2 CT/MRI AI

Possible approach: MONAI-based or another suitable pretrained segmentation model for a narrowly defined use case.



AAkriti should not claim a specific segmentation performance target such as Dice ≥ 0.80 as achieved performance unless the model, dataset, task and validation protocol are explicitly defined and measured.

14. Doctor Verification and Safety Layer

Doctor verification is a central product feature. The system should not send raw AI output directly to the patient.

AI → Doctor → Patient

- APPROVE — AI result is acceptable for patient communication.
- EDIT — Doctor modifies or corrects the highlighted finding.
- REJECT — AI result is unsuitable and is not shown to the patient.

Safety principle: AI assists. Doctor decides.

15. Patient Experience

13. Your Scan — display the original imaging.

14. What Your Doctor Is Showing You — display the verified highlighted finding.

15. Where Is It? — show the relevant anatomical reference.
16. What Does It Mean? — provide doctor-approved plain-language explanation.
17. Explore — provide 3D/AR where available and appropriate.
18. Questions to Ask Your Doctor — optional educational prompts without autonomous medical advice.

16. Innovation and Differentiation

- Image-grounded — explanation is linked to the patient's actual imaging.
- Doctor-controlled — AI output is verified before patient presentation.
- Modality-aware — visualization changes according to X-ray, CT or MRI.
- Communication-first — designed around the doctor-patient conversation.
- Low-friction — web/mobile visualization can reduce dependence on specialized AR hardware.

16.1 Unique Value Proposition

AAkriti adds a doctor-controlled communication layer to existing medical-imaging workflows, allowing clinicians to turn verified findings from a patient's scan into understandable 2D, 3D and optional AR explanations without requiring specialized visualization hardware.

Short version: "Show the patient what the doctor sees."

Safety version: "AI assists. Doctor verifies. Patient understands."

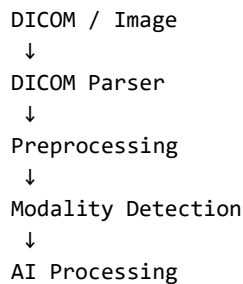
17. Technical Architecture

Layer	Technology / approach	Primary responsibility
Frontend	React + Vite	Doctor dashboard, upload, verification, patient view, 3D viewer
Backend	Python + FastAPI	APIs, uploads, processing, inference orchestration, case state
DICOM	pydicom + SimpleITK	Parsing, metadata, image/volume handling
Image processing	OpenCV + NumPy	Preprocessing and image operations
X-ray AI	YOLO / suitable pretrained model	Localization/detection
CT/MRI AI	MONAI / suitable pretrained model	Segmentation
3D	Three.js + WebGL	Interactive model rendering
AR	WebXR / WebAR-compatible approach	Optional mobile/web AR
Database	MongoDB Atlas or local MVP	Case metadata, verification state, notes and asset references
Storage	Secure local/object storage; media storage for generated assets if appropriate	Visualization assets

18. Data and Storage Architecture

18.1 DICOM / Image Ingestion

DICOM/image ingestion should be treated as an early part of the pipeline, not as a side feature.



18.2 Database

- Case ID
- Patient/case reference
- Modality
- AI result
- Confidence
- Doctor approval status
- Doctor notes
- Patient-facing explanation
- Visualization asset references
- Timestamp

18.3 Cloudinary Role

If Cloudinary is retained in the prototype, its role should be limited to storing/serving generated visualization assets such as highlighted images and 3D .glb models. It is not the AI model store. For real healthcare deployment, secure healthcare-compatible storage and compliance requirements must be evaluated before storing patient data in any external service.

19. Data Strategy

19.1 Imaging Data

- X-ray
- CT
- MRI
- DICOM studies

19.2 Potential Data Sources

- Public medical imaging datasets

- De-identified hospital data after appropriate approvals
- Research datasets
- Manually annotated datasets

The project must not claim access to hospital patient data unless such access has actually been obtained with appropriate authorization.

20. MVP Scope

20.1 Initial MVP Focus

Selected orthopedic/fracture cases.

20.2 X-ray MVP

- Upload image
- AI localization
- Highlighted region
- Doctor verification
- Patient-facing explanation

20.3 CT Demonstration

- One selected demonstrable case
- DICOM volume
- Segmentation
- 3D model
- Doctor verification
- Interactive 3D viewer

20.4 AR

Optional demonstration on a validated prepared case.

21. 36-Hour Execution Plan

Time	Work
0–8 hours	React UI, FastAPI, sample/de-identified cases, upload
8–18 hours	X-ray AI inference, highlighting and doctor verification
18–28 hours	Selected CT case, segmentation and 3D viewer
28–32 hours	Patient explanation interface
32–36 hours	Integration, testing, demo, PPT and Q&A preparation

21.1 Explicitly Not Attempted

- Training a new medical model from scratch

- All fracture types and all diseases
- Universal automatic X-ray-to-patient-specific-3D reconstruction
- Production PACS integration
- Treatment recommendations
- Healing-time prediction
- Complete clinical-grade regulatory deployment
- Critical dependence on external APIs

22. AI Underperformance and Fallback Strategy

Failure	Response
AI false detection	Doctor verification, editing or rejection
Limited dataset	Narrow MVP, public/de-identified data, gradual validation
Poor segmentation	Do not show misleading 3D; fall back to verified 2D
X-ray depth ambiguity	Use 2D-first explanation
API/model failure	Local or preloaded inference and prepared demo cases
3D reconstruction error	Quality checks; fallback to 2D
Clinical misuse	Communication aid positioning; doctor remains decision-maker

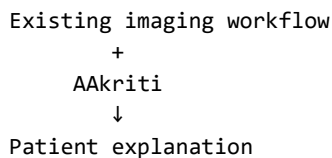
23. Feasibility and Viability

23.1 Technical Feasibility

The core building blocks—DICOM processing, AI image analysis, segmentation, 3D reconstruction, WebGL and web/mobile visualization—already exist. The main engineering challenge is integrating them into a reliable and safe communication workflow.

23.2 Operational Feasibility

AAkriti is designed as an add-on communication layer rather than a replacement for PACS/RIS.



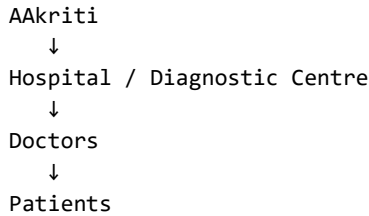
23.3 Financial Feasibility

The prototype can use local development and open-source components. Production costs will depend on infrastructure, storage, security, clinical validation, integration and regulatory requirements.

24. Business Model

24.1 Model

B2B / B2B2C



24.2 Revenue Streams

- Institutional SaaS subscription
- Per-case pricing
- Enterprise licensing for diagnostic chains
- PACS/HIS integration fees
- White-label deployment

24.3 Pricing Hypothesis

Earlier research suggested illustrative pricing of approximately ₹5,000–₹10,000/month for a basic institutional plan and ₹50,000+ for enterprise configurations. These are pricing hypotheses, not validated market prices, and should be tested through customer interviews and pilots.

25. Market Opportunity

Earlier research used Indian diagnostic imaging services and healthcare AR market estimates as broad market context. These figures should not be treated as AAkriti's exact TAM without a bottom-up calculation.

Market context	Earlier research estimate	Use
Indian diagnostic imaging services	Approximately USD 6.33B in 2024; projected approximately USD 11.55B by 2032; CAGR ~7.8%	Broad market context
Indian AR in healthcare	Approximately USD 81.53M in 2024; projected approximately USD 529.53M by 2033; CAGR ~21.4%	Technology/market context

These are market-research estimates from the earlier project research and should be cited to their original market source when used externally.

25.1 TAM Approach

A stronger future TAM calculation should use a bottom-up model: number of applicable institutions × relevant cases/year × potential AAkriti revenue per institution or case. Diagnostic-imaging and AR markets should not simply be added because they overlap.

25.2 SAM

The serviceable market is the subset of hospitals, diagnostic centres and clinics that have digital imaging workflows and a need or willingness to adopt patient-facing imaging communication.

25.3 SOM

The earlier research used 0.5–1% of SAM over 3–5 years as an illustrative adoption scenario, beginning with Nagpur and selected Indian cities. This is a scenario, not a forecast.

26. Target Market and Go-To-Market Strategy

26.1 Target Institutions

- Hospitals
- Diagnostic centres
- Orthopedic clinics
- Radiology departments
- Diagnostic chains

26.2 Geographic Expansion

19. Nagpur pilot
20. Other Maharashtra cities
21. Major Indian cities
22. National diagnostic chains
23. International markets after validation and regulatory readiness

26.3 GTM Phases

Phase	Strategy
Phase 1 — Pilot	Small number of hospitals/diagnostic centres; measure patient comprehension and doctor usability
Phase 2 — Regional/National	Approach diagnostic chains and hospital groups
Phase 3 — Ecosystem	Integrate with PACS, HIS, EHR, patient portals and telemedicine platforms

27. Competitive Landscape

AAkriti operates in an existing ecosystem. Competitors and adjacent systems may provide DICOM processing, segmentation, 3D visualization, AI analysis, AR/VR, patient sharing or patient communication.

Claims such as “zero competitors,” “first,” or “no existing system does this” should not be used without a comprehensive current market review.

Category	Examples / context	AAkriti positioning
Medical 3D platforms	3D Slicer, Materialise Mimics, Simpleware, OsiriX, RadiAnt	Communication layer rather than replacement
AR/VR healthcare	Anatomy, planning and education systems	Low-friction patient communication
Patient-focused imaging platforms	DICOM/3D/AI/patient-sharing products	Doctor-controlled, image-grounded workflow
Academic multimodal systems	Systems combining simplification, segmentation, images and 3D	Focused consultation workflow and verification

27.1 Competitive Positioning Statement

AAkriti does not try to replace existing medical imaging viewers; it adds a doctor-controlled communication layer that turns a verified finding from the patient's scan into an understandable visual explanation.

28. SWOT Analysis

Strengths	Weaknesses
Patient-centric communication	Limited initial clinical validation
Established technology components	AI errors and image-quality dependence
Doctor-controlled workflow	Segmentation limitations
Low specialized-hardware requirement	Integration complexity
SaaS scalability	Clinician adoption requirements
Image-grounded and modality-aware	Narrow initial use case

Opportunities	Threats
Growing medical imaging	Established competitors
AI-assisted radiology	Regulatory requirements
Patient experience and education	Privacy/security requirements
Diagnostic-chain differentiation	Workflow resistance
Telehealth and digital health	Cost sensitivity
PACS/HIS/EHR integration	Model generalization problems

29. Risk Management

Risk	Potential impact	Mitigation
AI false detection	Incorrect patient-facing visualization	Doctor verification + manual correction/rejection
Limited/diverse data	Poor generalization	Narrow scope + public/de-identified data + future external

		validation
Poor segmentation	Misleading 3D model	Quality checks + 2D fallback
X-ray depth ambiguity	False precision in 3D	2D-first X-ray strategy
Privacy breach	Patient-data risk	De-identification, access control, encryption and controlled sharing
External API failure	Demo/system failure	Local/preloaded inference
Clinical misuse	Unsafe interpretation	Communication-only positioning + doctor in loop
Integration complexity	Adoption friction	API-first architecture + phased integration

30. Privacy, Security and Regulatory Considerations

30.1 Privacy Principles

- Use de-identified data for prototype demonstrations.
- Minimize unnecessary patient information.
- Use role-based access control.
- Authenticate doctors and authorized users.
- Use secure transmission and storage.
- Provide controlled patient sharing.
- Avoid exposing raw AI outputs directly to patients.

30.2 India Regulatory Consideration

Because AAkriti involves medical imaging and software used around clinical workflows, regulatory classification and compliance requirements must be assessed before commercial clinical deployment. India's medical-device framework includes software intended for specified medical purposes within the broader medical-device regulatory context.

For the MVP, AAkriti should be positioned as a clinician-controlled communication and visualization aid rather than an autonomous diagnostic or treatment system. Commercial deployment should involve appropriate regulatory, clinical, privacy, cybersecurity and quality-management assessment.

31. Impact Analysis

31.1 Patient Impact

- See the actual finding.
- Understand its location.
- Connect technical information with visual anatomy.
- Participate more meaningfully in the consultation.

31.2 Doctor Impact

- Use a shared visual reference.

- Point to the relevant finding.
- Explain using image + visualization.
- Verify AI before patient presentation.

31.3 Healthcare Institution Impact

- Standardize patient-facing imaging communication.
- Offer a digital patient-education layer.
- Potentially differentiate patient experience.
- Scale the workflow across locations.

31.4 Social Impact

Potentially improves the accessibility and clarity of medical-imaging explanations for patients and caregivers.

31.5 Economic Impact

Creates a potential B2B SaaS opportunity and may streamline repetitive explanation workflows; specific savings must be measured.

31.6 Environmental Impact

Digital explanations may reduce reliance on printed educational material and physical 3D models in some workflows; specific reductions require measurement.

32. SDG Alignment

SDG	Alignment
SDG 3 — Good Health and Well-being	Primary alignment through patient communication and healthcare understanding
SDG 4 — Quality Education	Secondary alignment through patient and medical education
SDG 9 — Industry, Innovation and Infrastructure	Secondary alignment through digital healthcare technology and infrastructure

33. Validation Framework

AAkriti should eventually be evaluated against the conventional communication workflow rather than only demonstrating that the software works technically.

Area	Metric
Patient	Comprehension score
Patient	Recall of key information
Patient	Satisfaction / usability
Patient	Questions remaining after explanation
Doctor	Explanation time
Doctor	AI correction frequency
Doctor	Perceived usefulness and usability

AI detection	Sensitivity, specificity, precision, recall, F1/AUROC where appropriate
Segmentation	Dice, IoU and relevant geometric metrics
System	Processing time, latency and visualization success rate

33.1 Experimental Comparison

Control:

2D imaging + normal verbal explanation

Intervention:

AAkriti visual explanation

2D highlight

+

3D/AR where appropriate

+

Doctor-controlled explanation

The objective is to determine whether the AAkriti workflow improves patient understanding without creating unacceptable additional burden for clinicians.

34. Success Criteria

The following are proposed validation targets and engineering goals, not current AAkriti results.

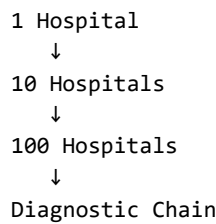
- Patients demonstrate measurable improvement in comprehension compared with the baseline workflow.
- Doctors can use the system without a meaningful increase in consultation burden.
- AI achieves predefined performance thresholds for each narrowly defined use case.
- Segmentation quality is sufficient for safe visualization in selected CT/MRI use cases.
- Visualization generation is reliable within the intended consultation workflow.
- Patient-facing output is shown only after clinician verification.

35. Product Roadmap

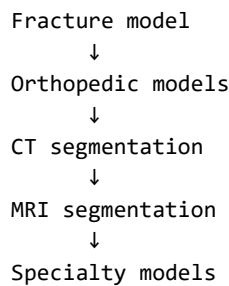
Phase	Scope
Phase 1	Selected fractures; X-ray 2D highlighting; selected CT 3D demonstration
Phase 2	Selected orthopedic conditions
Phase 3	Additional validated CT use cases
Phase 4	Selected MRI use cases
Phase 5	Multiple medical specialties
Phase 6	PACS / RIS / HIS / EHR integration and multi-centre deployment

36. Scalability Strategy

36.1 Institutional Scalability



36.2 AI Scalability



36.3 Technology Scalability

- Cloud/SaaS deployment
- Modular AI services
- API-first architecture
- Web/mobile visualization
- Centralized software updates
- Multi-site administration
- Future PACS/RIS/HIS/EHR integration

37. Intellectual Property and Innovation Strategy

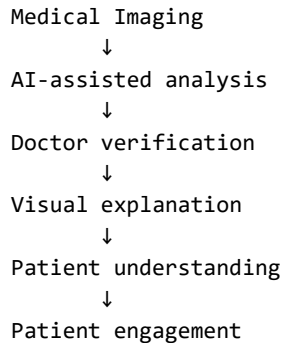
Potentially protectable elements may include aspects of the workflow architecture, doctor-verification mechanism, modality-aware visualization workflow, communication interface and system integration methodology. Patentability or freedom-to-operate should not be claimed without a formal prior-art and patent search.

38. Sustainability

- Recurring SaaS revenue.
- Modular AI architecture.
- Cloud or on-premise deployment options.
- Expansion to additional modalities and specialties.
- Integration with existing healthcare infrastructure.
- Institutional licensing and enterprise plans.

39. Long-Term Vision

AAkriti can evolve from a tool for visual explanation of individual imaging findings into a standardized visual communication layer for medical imaging.



40. Five Things to Remember

- 24. The problem is not diagnosis; it is understanding.
- 25. The explanation is grounded in the patient's own medical imaging.
- 26. AI assists; the doctor verifies.
- 27. Visualization depends on modality: X-ray → 2D; CT/MRI → 3D/AR where suitable.
- 28. AAkriti adds a communication layer to existing imaging workflows rather than replacing them.

41. Final One-Liners

AAkriti turns a verified medical-imaging finding into an understandable visual explanation for the patient.

AAkriti helps doctors show patients exactly what they mean when explaining a medical-imaging finding.

"Show the patient what the doctor sees."

"AI assists. Doctor verifies. Patient understands."

42. Key References to Include in the Final Version

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- 2) van der Mee et al. (2025). Systematic review of radiology report formats and patient understanding.
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